

CANONICAL LESSON

V7_B_N12 - Health Surveys and Small-Area Evidence

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C32
Notebook	V7_B_N12
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Use health surveys and small-area evidence with valid design, weights, precision, model assumptions, and disclosure safeguards.

Learning outcomes

Why this matters

Small-area precision can be overstated when design, sparsity, or model dependence is hidden.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

District estimates combine survey design evidence with transparent uncertainty and suppression rules.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Compare weighted and unweighted prevalence.
- 2.
- Add effective sample size.
- 3.

- Explain model-assisted release safeguards.

Answer key

- 1.
- Weighting represents unequal selection and response; disagreement is a diagnostic, not a reason to discard weights.
- 2.
- Use squared-weight concentration as a simple diagnostic and the full design for official variance.
- 3.
- Pre-specify models, validate domains, publish uncertainty, protect confidentiality, and retain official release review.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.